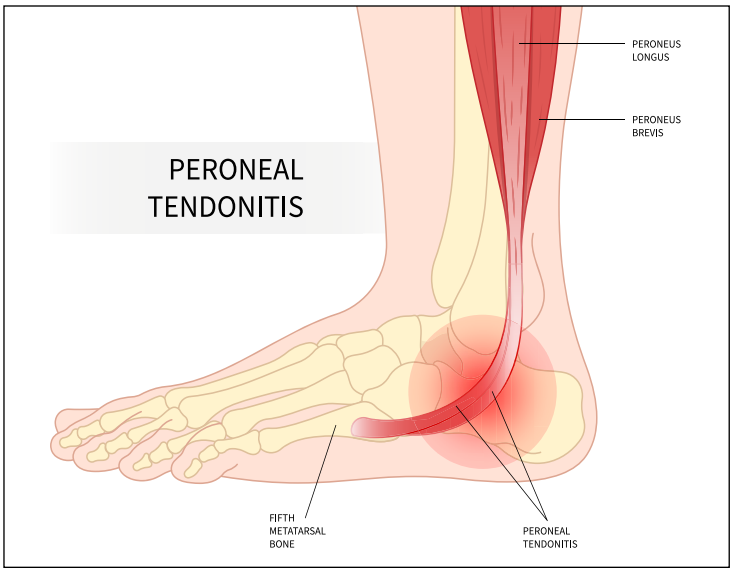
Posterior Tibial Tendinopathy

Background

- Posterior tibial tendinopathy is a common overuse injury affecting about 10% of Americans, both athletes and non-athletes alike.
- The posterior tibial tendon (PTT) is on the inner side of the ankle and it helps the foot point downand in, while also stabilizing the arch.
- People who rapidly increase their exercise intensity, who have weakness in their core foot muscles , hypertension, diabetes, obesity, inflammatory conditions such as rheumatoid arthritis, are over age 40, or women, are all at higher risk for developing posterior tibial tendinopathy.
- Symptoms typically include pain along the inside of the foot and ankle and difficulty performing a heel raise.
- Diagnosis is made through patient history and physical examination. Additional medical imaging, such as x-rays, ultrasound, and MRI may be used to confirm the diagnosis.
- Primary treatment focuses on rest, activity modification, and rehabilitation with particular emphasis on foot and ankle strengthening exercises. Additional treatments may involve weight loss, temporary use of orthotics for arch support, and injections .
- Most cases improve within 3–6 months with proper rehabilitation. In some severe cases, the tendon can rupture and surgical repair is required.

Latest Research

- Corticosteroid injections **are not recommended** for managing posterior tibial tendinopathy due to their association with poor long-term outcomes and an increased risk of tendon rupture.
- Prolotherapy or platelet-rich plasma (PRP) injections may be considered if pain and dysfunction continues for more than 3 months of conservative treatment that includes formal physical therapy, orthotics, and weight loss.
- Surgery may be necessary if conservative treatments and injections are ineffective or if the tendon ruptures.



Rehab Instructions

Foot Doming

- Stand with your feet shoulder-width apart.
- Keep your toes flat on the floor (don't curl them) and pull the ball of your foot toward your heel, "doming" the arch of your foot.
- Hold for 6 seconds.

Complete 2 sets of 15 reps 2 x a day.





Toe Yoga





- Stand with your feet shoulder-width apart.
- First, lift up your big toe while keeping your other 4 toes on the floor. Then, lower your big toe back to the floor.
- Next, press your big toe into the floor and lift the other 4 toes. Then, lower your toes back to the floor.
- Hold each position for 6 seconds.

Complete 2 sets of 15 reps 2 x a day.

Resisted Windshield Wipers





- Use a looped resistance band or tie a knot in a resistance band to create a loop. Tug on the knot to make sure it's secure.
- Sit in a chair and loop the resistance band around your foot, then anchor it to something sturdy such as the leg of a table or couch.
- Keep your heel in contact with the floor and slowly move your foot outward (abduction) then inward (adduction) against the resistance band.
- Keep the movement isolated to your foot. Don't rotate your knee or hip.

Complete 3 sets of 15 reps, 2 x a day.

Practical Tips for Success

- Tendons take time to heal. Full recovery may require 3 or more months.
- Progressive tendon loading is essential for effectively treating tendinopathy.
- For acute tendinopathy with swelling, a short course (7–14 days) of anti-inflammatory medications such as ibuprofen or naproxen may be beneficial.
- Immobilization with a short leg cast or a walking boot may be necessary for the first 2–3 weeks if walking causes significant pain.
- Orthotics can provide arch support and help off-load the tendon.
- You can complement your home rehab program with non-invasive treatments such as manual therapy, gait analysis, and addressing muscle weaknesses in your hips and pelvic core.

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